

Dynamic Psychopathology

1. Defence Mechanisms

Defence mechanisms are explanatory (not descriptive) psychopathology — they explain behaviour. They operate in both normal individuals and under pathological conditions.

How a Defence Mechanism is Formed

Formation Schema

Wish / Impulse → (Moral/Social/Legal Prohibitions) → Signal Anxiety → Defence Operation → Symptoms Formed

Classification by Vaillant (1977)

Category	Key Feature	Examples
Mature	Healthy; seen in well-adjusted adults	Altruism, Humour, Anticipation, Sublimation, Suppression
Neurotic	Mental inhibition; insight retained	Repression, Displacement, Reaction Formation, Rationalisation, Isolation, Intellectualisation, Dissociation, Undoing
Narcissistic / Psychotic (Kleinian)	Distort reality significantly	Projection, Denial, Splitting, Projective Identification, Omnipotence, Grandiosity
Immature	Normal in childhood; pathological if persistent in adults	Acting Out, Passive Aggression, Somatisation, Regression

MNEMONIC: SASHA = Mature defences

Sublimation · Altruism · Suppression · Humour · Anticipation

MNEMONIC: SIPDOG = Kleinian defences

Splitting · Introjection · Projective identification · Denial · Omnipotence · Grandiosity

A. Mature Defences (SASHA)

Defence	Process	Clinical Hook
Sublimation	Channelling unacceptable impulse into socially acceptable behaviour — instinct rechannelled, not blocked	Doctor with aggressive drives becomes a surgeon
Altruism	Vicarious satisfaction through service to others — does NOT require giving up one's own pleasures (contrast: altruistic surrender)	Volunteering after a personal loss

Defence	Process	Clinical Hook
Suppression	Conscious/semi-conscious postponement of attention to conflict — discomfort acknowledged but minimised	Not unconscious like repression — the key distinction
Humour	Express troublesome feelings overtly with comedy — tolerates painful aspects without unpleasant effect on others	Gallows humour in medics
Anticipation	Goal-directed planning for anticipated future difficulty — not free-floating anxiety	Pre-operative counselling

⚠ EXAM TRAP: Altruism vs Altruistic Surrender: In Altruism the person still enjoys vicarious satisfaction. In Altruistic Surrender, one completely gives up own needs to serve others — this is NOT a mature defence.

⚠ EXAM TRAP: Suppression vs Repression: Suppression is CONSCIOUS postponement — the person knows the thought exists. Repression is UNCONSCIOUS — the thought is banished without awareness.

B. Neurotic Defences

Operate at the level of mental inhibition. Insight is retained but some freedom in decision-making is lost.

Defence	Key Mechanism	Classic Association
Repression	Unconscious expulsion of threatening ideas/feelings from consciousness. Primary = never reached consciousness; Secondary = was conscious then expelled. The PRIMARY defence.	Hysteria; underlies all other defences
Displacement	Shift of emotion from threatening object to a less threatening substitute — cathexis transferred	Phobias; anger at boss displaced onto spouse
Reaction Formation	Unacceptable impulse transformed into exact opposite. Becomes character trait if used early in development.	OCD / obsessional personality; excessive cleanliness over underlying mess impulses
Isolation (of Affect)	Idea is separated from its accompanying affect — the affect is repressed while idea remains conscious	OCD obsessions; discussing trauma without emotion
Rationalisation	False but socially acceptable explanations for unacceptable behaviour driven by instinctual impulses	Alcohol misuse; teenage conduct problems
Intellectualisation	Excessive use of intellectual/abstract reasoning to avoid emotional	Focusing on inanimate facts; avoiding intimacy

Defence	Key Mechanism	Classic Association
	experience — no instinctual drive involved	
Dissociation	Drastic temporary modification of personal identity/consciousness to avoid emotional distress	Fugue, conversion disorder, multiple personalities, counter-phobic behaviour
Undoing	Symbolic negation of unacceptable impulse through ritual/magical thinking	OCD compulsions; tapping rituals
Identification with Aggressor	Victim assumes qualities of the aggressor to reduce direct resistance during acute trauma	Stockholm syndrome; child abuse survivors

Intellectualisation vs Rationalisation — High-Yield Comparison

Intellectualisation	Rationalisation
NO instinctual impulses/drives involved	Instinctual impulses/urges ARE involved
Avoid EXPERIENCING unpleasant affect entirely	Might experience affect but reduces its impact
Deals with inanimate facts and details instead of feelings	Provides 'excuses' — e.g. for drinking, teenage conduct

Repression vs Dissociation — High-Yield Comparison

Repression	Dissociation
Archaeological (vertical) storage — varying depths	Horizontal storage — all units equally accessible
Motivated forgetting	Amnesic barriers maintain dissociation
Scattered across time (dynamic conflicts)	Discrete and delimited in time
Information transformed and disguised	Untransformed storage
Requires repeated trials + interpretation	Direct retrieval e.g., hypnosis
Therapy: transference work and working through	Therapy: integration of memories, working through trauma

C. Kleinian (Narcissistic / Psychotic) Defences (SIPDOG)

Defence	Key Mechanism	Clinical Association
Splitting	Qualities split into all-good or all-bad — no grey area. Leads to idealisation alternating with denigration.	Borderline PD — most characteristic
Idealisation & Denigration	Object is either glorified/omnipotent OR cursed/devalued. Accompanies splitting.	Borderline PD; common in therapeutic relationships
Projective Identification (PI)	Part of self is projected onto another, who is then pressured to identify with it. Projector then sees their own hostility	Psychotic paranoid states; borderline PD

Defence	Key Mechanism	Clinical Association
	as coming from the other. 3 steps (Ogden).	
Introjection	Internalising qualities of an external object (e.g. mother)	Normal development; seen in grief
Omnipotence	Belief that thoughts alone can influence the external world (magical thinking)	OCD — distressing intrusive thoughts; narcissistic features
Grandiosity	Converting inferiority feelings to superiority — manic defence	Narcissistic PD; mania

Ogden's 3 Steps of Projective Identification

Step	Name	What Happens
1	Projection	Part of self projected onto external object (Step 1a: blurring of self and object representations — may or may not be present)
2	Interpersonal pressure	Projector actively pressures recipient to think, feel, act in line with projection (NOT required for simple 'projection')
3	Re-internalisation	Projection is reinternalised after being processed by recipient (ABSENT in simple projection)

⚠ EXAM TRAP: Projection vs Projective Identification: In simple Projection — only Step 1 occurs. In PI — Steps 2 and 3 are added. Step 3 is absent in simple projection. The recipient's actual behaviour is influenced in PI but not in projection.

D. Immature Defences

Defence	Mechanism	Clinical Link
Acting Out	Unconscious wish expressed through action to avoid conscious affect — impulsive wish-fulfillment	Dissocial PD; substance misuse; borderline PD
Passive Aggression	Aggression expressed indirectly through passive obstruction (procrastination, sick leave)	Personality disorders; workplace dysfunction
Somatisation	Psychological tension converted to bodily symptoms	Somatoform disorders; medically unexplained symptoms
Regression	Return to earlier developmental stage to avoid confronting conflict. Also linked to creative process.	Depression; hospitalised patients
Somatosensory Amplification	Tendency to experience bodily sensations as unusually intense — underpins somatoform disorders	Health anxiety; somatoform disorders

⚠ EXAM TRAP: Denial vs Conscious Avoidance: Denial operates at the UNCONSCIOUS level — the person genuinely cannot acknowledge the reality. It is NOT the same as consciously avoiding a painful topic or thought. Conscious avoidance = aware but choosing not to engage. Denial = unaware at an unconscious level.

Defences and Disorders — High-Yield Table

Disorder	Defences Commonly Used
Alcoholism	Denial, Rationalisation
Anorexia Nervosa	Denial, Rationalisation
Borderline PD	Splitting, Idealisation, Denigration, Projection, Dissociation, Acting Out
Depression	Regression
Dissocial PD	Acting Out
Fugue / Amnesia	Dissociation
Hysteria (Conversion)	Repression, Conversion
OCD	Isolation of Affect, Undoing, Reaction Formation, Magical Thinking
Paranoid Delusions	Projection
Phobia	Displacement, Avoidance
Schizoid PD	Fantasy, Avoidance
Somatoform Disorders	Somatisation
Narcissistic PD	Projection, Splitting

2. Dynamic Models of the Mind

A. Freud's Topographical Theory (1900)

Published in 'The Interpretation of Dreams'. Divides mind into three regions. Based on Pleasure Principle vs Reality Principle.

System	Contents / Features	Key Point
Conscious	Receives/processes external world info; communicated via speech & behaviour; attention cathexis (investment of psychic energy)	Secondary process thinking dominant; operates on Reality Principle
Preconscious	Available on demand; interfaces between unconscious and conscious; maintains 'repressive barrier'	Does NOT contain repressed material — only censors access to it
Unconscious	Repressed wishes; primary process thinking; pleasure principle; rapid cathexis shifts	Evident via parapraxes (Freudian slips) and dreams

Limitations of Topographical Theory

- Defence mechanisms (e.g. repression) are themselves unconscious — cannot be in preconscious, which is available to consciousness
- Cannot explain unconscious need for punishment noted in patients

B. Instinct / Drive Theory

Characteristic	Description
Source	Part of body where instinct originates
Impetus	Intensity / force of the instinct
Aim	Action directed towards discharge of energy/tension
Object	Target for action

MNEMONIC: SIAO = 4 characteristics of an instinct

Source · Impetus · Aim · Object

Concept	Eros (Life Instinct)	Thanatos (Death Instinct)
Drive	Sexual / Libido	Aggression / Destruction
Origin	Libido = force of sexual instinct in mind	Originates in skeletal muscles
Dominant force?	No	YES — dominant force in biology per Freud

 **EXAM TRAP: Dominant instinct: According to Freud, THANATOS (death instinct) is the DOMINANT force in biology — not Eros. This is frequently tested.**

C. Hierarchy of Anxiety (Developmental Sequence)

Anxiety types in developmental order from most primitive to most mature:

#	Anxiety Type	Key Feature
1	Signal Anxiety	Unconscious perception of threat → resource mobilisation → defence mechanisms. Forms the BASIS of defence mechanisms.
2	Disintegration / Annihilation Anxiety	Concern about fusion with external object (most primitive)
3	Stranger Anxiety	Appears ~7–9 months of age
4	Separation Anxiety	Appears when mother is recognised as independent object
5	Fear of Object Loss / Loss of Love	Especially in girls at the phallic stage

#	Anxiety Type	Key Feature
6	Castration Anxiety	Phallic stage; especially in boys
7	Superego Anxiety	Most MATURE form — id vs ego conflicts; conscience-driven guilt

MNEMONIC: Hierarchy of Anxiety mnemonic: 'Silly Donkeys Should Fear Castrating Super-egos'

Signal · Disintegration · Stranger · Separation · Fear of loss · Castration · Superego

D. Analytical Psychology — Jungian Model

Jungian Concept	Definition
Collective Unconscious (CU)	All mankind's collective symbolic past — like a 'psychological DNA'. Must be differentiated from Personal Unconscious.
Personal Unconscious (PU)	Equivalent to Freudian unconscious — collection of repressed individual memories
Archetypes	Part of CU — representational images with universal symbolic meaning (Hero, Old Wise Man, Shadow, etc.)
Complexes	Part of PU — feeling-toned ideas arising from interactions of complexes with archetypes via interpersonal experience
Persona	The 'mask' — personality presented to the outside world
Anima	Unconscious FEMININE aspect of a man
Animus	Unconscious MASCULINE aspect of a woman
Shadow	Archetype — personification of unacceptable aspects of self; symbolised as a dark internal alien
Individuation	Ultimate goal of life — development of true self-identity
Extraversion / Introversion	Terms INTRODUCED by Jung (not Freud)

⚠ EXAM TRAP: Anima vs Animus: Anima = feminine aspect of a MAN. Animus = masculine aspect of a WOMAN. These are frequently reversed in exam questions.

⚠ EXAM TRAP: Archetypes are part of the COLLECTIVE unconscious. Complexes are part of the PERSONAL unconscious. Do not confuse.

3. Dynamic Interpretation of Dreams

Historical Context

- Anna O. (Bertha Pappenheim) — first key case; treated by Breuer & Freud; led to hypnosis as technique
- Cathartic method (abreaction): recovering and verbalising suppressed feelings that caused symptoms

- Freud abandoned abreaction when patients couldn't recall memories (resistance) — led him to free association
- Dreams arose during free association — Freud declared them the 'royal road to the unconscious'
- Wish Fulfilment Theory: dreams are surrogate attempts to fulfil unconscious wishes

Dream Content Types

Content Type	Description
Manifest Content	What the dreamer actually RECALLS — the surface story
Latent Content	Underlying UNCONSCIOUS wishes and thoughts that threaten to awaken the dreamer
Dream Work	The unconscious mental operation that TRANSFORMS latent into manifest content

Dream Work Mechanisms

Mechanism	Definition	Example / Key Point
Condensation	Multiple unconscious impulses combined into ONE manifest image	Father + cruel teacher → single monster in child's dream
Irradiation / Diffusion	CONVERSE of condensation — one unconscious impulse represented by multiple manifest images	Opposite process to condensation
Displacement	Transfer of affective energy (cathexis) from original object to symbolic substitute — not mere substitution	Not just symbolic swap; involves transfer of emotional charge
Symbolic Representation	Charged or abstract concepts represented by innocent images connected to the original	Intense dancing representing sexual desire toward a colleague
Secondary Revision	More mature ego organises primitive dream content more coherently — secondary process	Logic, time and space intact; makes dream narrative 'sensible'

Primary vs Secondary Process Thinking

Primary Process (Id / Unconscious)	Secondary Process (Ego / Conscious)
Defies logic	Logical and rational
No sense of time or space	Intact time and space boundaries
Accepts contradictory items simultaneously	Cannot hold contradictions
Often incoherent	Coherent and mature
Characteristic of condensation, displacement, symbolic representation	Characteristic of secondary revision

Special Dream Types

Dream Type	Explanation
Anxiety Dreams	Reflect a FAILURE in the protective function of dream-work mechanisms
Punishment Dreams	DEFY wish-fulfilment theory — Freud explained them as a compromise between conscience and repressed wish; an unconscious wish for punishment exists

⚠ EXAM TRAP: Punishment dreams appear to contradict Freud's wish-fulfilment theory. The exam favourite explanation: they represent a **COMPROMISE** — the wish for punishment is itself an unconscious wish. They do NOT disprove the wish-fulfilment theory; Freud reconciled them.

MNEMONIC: Dream Work = CIDS + SR

Condensation · Irradiation (diffusion) · Displacement · Symbolic Representation · Secondary Revision

4. Examiner Favourites & High-Yield Traps Summary

Most Commonly Tested Distinctions

Topic	Concept A	Concept B
Suppression vs Repression	CONSCIOUS postponement; discomfort acknowledged	UNCONSCIOUS banishment; subject unaware
Repression vs Dissociation	Vertical 'archaeological' storage; scattered across time; motivated forgetting	Horizontal storage; discrete/delimited; amnesic barriers
Intellectualisation vs Rationalisation	No instinctual drive; avoids experiencing affect; inanimate focus	Instinctual drives involved; provides 'excuses'
Projection vs Projective Identification	Only Step 1 (projection); no interpersonal pressure; no reinternalisation	All 3 steps; recipient's behaviour actually influenced
Altruism vs Altruistic Surrender	Vicarious satisfaction retained — mature defence	Self is depleted to serve others — NOT a mature defence
Anima vs Animus	Anima = feminine aspect of a MAN	Animus = masculine aspect of a WOMAN
Archetypes vs Complexes (Jung)	Part of COLLECTIVE unconscious — universal symbolic images	Part of PERSONAL unconscious — feeling-toned ideas from experience
Eros vs Thanatos — dominant?	Eros = life instinct / libido	Thanatos = DOMINANT force in biology per Freud
Manifest vs Latent Content	What dreamer RECALLS	Underlying UNCONSCIOUS wishes
Condensation vs Irradiation	Many impulses → ONE image	ONE impulse → many images (opposite)

High-Yield Named Facts

- **SASHA** — mnemonic for mature defences (Sublimation, Altruism, Suppression, Humour, Anticipation)
- **SIPDOG** — mnemonic for Kleinian defences (Splitting, Introjection, Projective Identification, Denial, Omnipotence, Grandiosity)
- **Vaillant (1977)** — classified defences into mature, neurotic, and narcissistic/immature
- **Anna O. (Bertha Pappenheim)** — first psychoanalytic case; treated by Breuer & Freud; led to cathartic method
- **Freud:** Dreams = 'royal road to the unconscious'; Wish Fulfilment Theory
- **Stranger anxiety:** 7–9 months. Superego anxiety: most mature form of anxiety
- **Reaction formation:** Characteristic of obsessional neurosis; if used early, becomes permanent character trait
- **Undoing:** Magical thinking — seen in OCD. Associated with compulsive rituals
- **Repression:** The PRIMARY defence — all other defences reinforce it
- **Extraversion & Introversion:** Terms INTRODUCED by Jung, not Freud
- **Ogden's model of PI:** Step 3 (reinternalisation) is absent in simple projection; Step 2 is not necessary to define projection alone